



REFERRAL LABS
Revolutionizing Healthcare



NABL M(EL)T 00417

nanocard
Unlimited Testing



Patient Name : MR. SREERAM RAJU

Age / Gender : 74 years / Male

Mobile No. : 9945211059

Patient ID : 36605

Source : HOME COLLECTION

Referral : SELF

Collection Time : Oct 16, 2024, 01:24 p.m.

Reporting Time : Oct 16, 2024, 05:47 p.m.

Sample ID :



000829024

Test Description	Value(s)	Unit(s)	Reference Range
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Thyroid Function Test - Total

Thyroid Function Test

T3-Total Method : CLIA	1.963	ng/mL	0.8-1.9
T4-Total Method : CLIA	8.286	ng/dL	5.0-13.0
TSH-Thyroid Stimulating Hormone Method : CLIA	2.815	µIU/ml	0.35-4.75
Note :			

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Vitamin D3 (25 - OH)

Vitamin D (25 - Hydroxy) Method : CLIA	32.022	ng/mL	Children aged 0-14 years: <12ng/mL as vitamin D deficiency, 12-20ng/mL as vitamin D insufficiency, 20-100ng/mL as vitamin D sufficiency, >100ng/mL as vitamin D overload. Adults: <20ng/mL as vitamin D deficiency; 20ng/mL~30ng/mL as vitamin D insufficiency; 30ng/mL~100ng/mL as vitamin D sufficiency; >100ng/mL as vitamin D overdose.
Note	source for the reference range is Endocrine Society		

Interpretation:

Useful for :

Diagnosis of vitamin D deficiency .

Differential diagnosis of causes of rickets and Osteomalacia . Monitoring vitamin D replacement therapy . Diagnosis of hypervitaminosis D .

Vitamin D levels may vary according to factors such as geography, season, or the patient's health, diet, age, ethnic origin, use of vitamin D supplementation or environment.

Some potential interfering substances like rheumatoid factor, endogenous alkaline phosphatase, fibrin, and proteins capable of binding to alkaline phosphatase in the patient sample may cause erroneous results in immunoassays. Carefully evaluate the results of patients suspected of having these types of interferences.

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CRP-(C-Reactive Protein) Quantitative

CRP (C-REACTIVE PROTEIN)* Method : Immuno-turbidimetric	2.2	mg/L	0-6.0
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Interpretation

- Elevated test results can occur in patients with hypertension, elevated body mass index, metabolic syndrome/ diabetes mellitus, chronic infection (e.g., gingivitis, bronchitis), chronic inflammation (e.g. rheumatoid arthritis) and low HDL/high triglycerides
- Cigarette smoking can cause increased levels.

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3. Decreased test levels can result from moderate alcohol consumption, weight loss, and increased activity or endurance exercise

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Renal Function Test (RFT)

Renal Function Test

Urea Method : Urease-GLDH	34.3	mg/dL	Adult Male <50 yrs : 19-44 , >50 yrs : 18-55 Female <50 yrs : 15-40 , >50 yrs : 21 - 43
Creatinine Method : ENZYMATIC	0.82	mg/dL	Children(1 yrs -14 yrs) : 0.30 - 0.70 Adult Male : 0.72 - 1.25 Adult Female : 0.57 - 1.11
Uric Acid Method : Uricase	4.8	mg/dL	3-5-7.0
Serum Calcium Method : ARSENAZO	8.4	mg/dl	8.4 - 10.2

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Electrolytes

Electrolytes

Sodium Method : Indirect ISE	141.3	mmol/L	136 - 145
Potassium Method : Indirect ISE	4.66	mmol/L	3.5 - 5.1
Chloride Method : Indirect ISE	101.6	mmol/L	96 - 108

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Iron (Total)

Serum Iron Method : Ferrozine method without deproteinization	152.3	µg/dl	65 - 175
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Bio. Ref. Interval. :

Males: 65 - 175

Females: 50 - 170

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Glycosylated Hemoglobin - HbA1c (H.P.L.C - METHOD)

HbA1c (Glycated Haemoglobin) , WHOLE BLOOD EDTA

HbA1c (Glycated Haemoglobin) Method : HPLC	5.6	%	<5.7
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Test Description	Value(s)	Unit(s)	Reference Range
Estimated Average Glucose Method : Calculated	114.02	mg/dL	-



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Test Description	Value(s)	Unit(s)	Reference Range
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Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Diagnosing Diabetes	
Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments : HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

Graph

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Blood Glucose - Fasting (FBS)

Glucose Fasting Method : Hexokinase	102.0	mg/dL	70 - 100
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Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	<100
Impaired fasting glucose	100 - 125
Diabetes	=>126

Reference :

American Diabetes Association Comment : Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note 1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus. 2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Lipid Profile

Lipid Profile

Cholesterol-Total Method : CHOD-PAP	118	mg/dL	Desirable: <= 200 Borderline High: 201-239 High: >= 239
Triglycerides Method : Glycerol Oxidase - Peroxidase	91.6	mg/dL	Normal: < 150 Borderline High: 150-199 High: 200-499 Very High: >= 500
Cholesterol-HDL Direct Method : PVS-PEGME	35.9	mg/dL	35.3-79.5
LDL Cholesterol Method : Calculated	63.78	mg/dL	Optimal: < 100 Near optimal/above optimal: 100-129 Borderline high: 130-159 High: 160-189 Very High: >= 190
Non - HDL Cholesterol, Serum Method : calculated	82.10	mg/dL	Desirable: < 130 Borderline High: 130-159 High: 160-189 Very High: > or = 190





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Test Description	Value(s)	Unit(s)	Reference Range
VLDL Cholesterol Method : calculated	18.32	mg/dL	6 - 38

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Vitamin B12

Vitamin B12 (Cyanocobalamin) Method : CLIA	168.115	pg/mL	183-822
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Note

source for the reference range is Kit Insert

Interpretation:

Vitamin B12, also known as cyanocobalamin, is a water soluble vitamin that is required for the maturation of erythrocytes and coenzyme form for more than 12 different enzyme systems. Groups at risk for vitamin B12 deficiency include those

(1) older than 65 years of age (2) with malabsorption (3) who are vegetarians (4) with autoimmune disorders (5) taking prescribed medication known to interfere with vitamin absorption or metabolism, including nitrous oxide, phenytoin, dihydrofolate reductase inhibitors, metformin, and proton pump inhibitors (6) infants with suspected metabolic disorders.

The most common cause of Vitamin B12 deficiency is pernicious anemia. Deficiency of Vitamin B12 is associated with megaloblastic anemia and neuropathy. Excess Vitamin B12 is excreted in urine. No adverse effects have been associated with excess vitamin B12 intake from food or supplements in healthy people.

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- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Diagnostic Lab issuing this report nor its employees/representatives assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

****END OF REPORT****

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Jayashree

Dr. Jayashree
Ph.D. in Biochemistry





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COMPLETE BLOOD COUNT

Hemoglobin (Hb)* Method : Cymeth Photometric Measurement	10.5	gm/dL	13.5-18.5
Erythrocyte (RBC) Count* Method : Electrical Impedence	3.64	mil/cu.mm	4.5 - 5.5
Packed Cell Volume (PCV)* Method : Calculated	32.5	%	40-54
Mean Cell Volume (MCV)* Method : Electrical Impedence	89.3	fL	76-100
Mean Cell Haemoglobin (MCH)* Method : Calculated	28.9	pg	25-32
Mean Corpuscular Hb Conc. (MCHC)* Method : Calculated	32.3	gm/dL	32-36
Total Leucocytes (WBC) Count* Method : Electrical Impedence	3650	cell/cu.mm	4500 - 11500
Neutrophils* Method : VCSn Technology	70	%	34 - 71
Lymphocytes* Method : VCSn Technology	20	%	20 - 45
Monocytes* Method : VCSn Technology	04	%	2 - 10
Eosinophils* Method : VCSn Technology	06	%	1 - 6
Basophils* Method : VCSn Technology	00	%	0 - 2
Platelet Count* Method : Electrical Impedence	265	10 ³ /ul	150-450

Test done on Automated five part cell Counter(WBC ,RBC,Platelet count by Impedence method , colorimetric method for Haemoglobin ,WBC differential by flow cytometry using laser technology other parameters are calculated).

Note :

Advice-Anemia profile and follow up.

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Shilpashree dixit

LIVER FUNCTION TEST (LFT)

Liver Function Test

Bilirubin - Total Method : Diazonium Salt	0.53	mg/dL	Adults : 0.2 - 1.2
Bilirubin - Direct Method : Diazo Reaction	0.25	mg/dL	0.0 - 0.5

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Test Description	Value(s)	Unit(s)	Reference Range
Bilirubin - Indirect Method : Serum, Calculated	0.28	mg/dL	0.1 - 1.0
SGOT (AST) Method : NADH without P-5-P	25.3	U/L	5 - 35
SGPT (ALT) Method : NADH without P-5-P	29.0	U/L	0 - 55
Alkaline Phosphatase-ALP	103	U/L	MALE: 1 - 12 Yrs-<500 12-15 Yrs <750 > 20 150 FEMALE: 1 - 12 Yrs <500 > 15 Yrs - 40-150
Total Protein Method : Biuret	5.74	g/dL	6.4 - 8.3
Albumin Method : BCG	4.02	g/dL	3.5 - 5.2
A/G Ratio Method : Calculated	2.34		1.0 - 1.8
Globulin Method : Calculated	1.72		2.3 - 3.5

Saved By : Irshad A

Result Submitted By : Irshad A

Result Approved By : Shilpashree dixit

ERYTHROCYTE SEDIMENTATION RATE - ESR

ESR - Erythrocyte Sedimentation Rate	40	mm/hr	0 - 10
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Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and lewis practical hematology

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Saved By : CHANDINI

Result Submitted By : CHANDINI

Result Approved By : Shilpashree dixit

DR Shilpashree Dixit
KMC Registration 77264
Consultant Pathologist





NAME : SREERAM RAJU (74Y/M)
REF. BY : SELF
TEST ASKED : AMA,ATG,FT3,FT4

SAMPLE COLLECTED AT :
NO. 39 & 40 GROUND FLOOR NCBS ROAD CANARA BANK
LAYOUT RAJIV GANDHI NAGAR SAHAKAR NAGAR POST
BENGALURU 560092 - 560092

PATIENTID : SR24930392

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
FREE TRIIODOTHYRONINE (FT3)	E.C.L.I.A	3	pg/mL	2.0-4.4
FREE THYROXINE (FT4)	E.C.L.I.A	1.36	ng/dL	0.93-1.7

Comments :

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

FT3 - Fully Automated Electrochemiluminescence Compititive Immunoassay,
FT4 - Fully Automated Electrochemiluminescence Compititive Immunoassay

THIS TEST HAS BEEN OUTSOURCED TO AN NABL LABORATORY

~~ End of report ~~

Sample Collected on (SCT) : 16 Oct 2024 15:58
Sample Received on (SRT) : 16 Oct 2024 17:11
Report Released on (RRT) : 16 Oct 2024 19:23
Sample Type : SERUM
Labcode : 1610094107/P5054
Barcode : CK946259

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

Page : 3 of 5



NAME : SREERAM RAJU (74Y/M)
REF. BY : SELF
TEST ASKED : AMA,ATG,FT3,FT4

SAMPLE COLLECTED AT :
 NO. 39 & 40 GROUND FLOOR NCBS ROAD CANARA
 BANK LAYOUT RAJIV GANDHI NAGAR SAHAKAR
 NAGAR POST BENGALURU 560092 - 560092

PATIENTID : SR24930392

TEST NAME	TECHNOLOGY	VALUE	UNITS
ANTI THYROGLOBULIN ANTIBODY (ATG) Bio. Ref. Interval. :-	C.L.I.A	< 0.9	IU/mL

NEGATIVE : < 4
 POSITIVE : > 4

Clinical Significance:

Thyroglobulin is a key protein in the thyroid gland essential for the synthesis of thyroxine (T4) and tri-iodothyronine (T3). In an autoimmune disorder antithyroglobulin antibodies are produced against the thyroglobulin which damage the thyroid gland. The presence of thyroid autoantibodies cause disorders such as Hashimoto's thyroiditis, Graves disease, hypothyroidism, thyroid cancer etc.

Specifications: Precision: Intra assay (%CV): 5.7%, Inter assay (%CV): 5.2%; Sensitivity: 0.9 IU/ml

Kit Validation reference: Burtis CA, Ashwood ER, editors, Tietz textbook of clinical chemistry 2nd-ed Philadelphia WB Sanders 1994.

Please correlate with clinical conditions.

Method:- TWO-STEP IMMUNOENZYMATIC (SANDWICH) ASSAY

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 BANK LAYOUT RAJIV GANDHI NAGAR SAHAKAR
 NAGAR POST BENGALURU 560092 - 560092

PATIENTID : SR24930392

TEST NAME	TECHNOLOGY	VALUE	UNITS
ANTI MICROSOMAL ANTIBODY (AMA)	C.L.I.A	0.8	IU/mL
Bio. Ref. Interval. :-			

Negative : < 9
 Positive : >=9

(AMA are also called as Anti-TPO Antibodies).

Clinical Significance:

Microsomes are found inside thyroid cells. The body produces antibodies against microsomes when there has been some damage caused to thyroid cells. Autoimmunity induces the immune system to produce antibodies against host tissue causing inflammation or impairing thyroid function. Presence of antimicrosomal antibody (AMA) is generally associated with autoimmune disorder. A positive test for AMA can indicate Hashimoto's thyroiditis or granulomatous thyroiditis or Rheumatoid arthritis or Autoimmune hemolytic anaemia (rarely) etc.

Specifications: Precision: Intra assay (%CV): 7.1%, Inter assay (%CV): 4.40%; Sensitivity: 0.25IU/mL

Kit Validation reference:

Kimura S Kotani T Mc Bride OW Umeki K Hirai Nakayama T. Human thyroid peroxidase complete cDNA and protein sequence chromosome mapping and identification of two alternately spliced mRNAs. Proc Natl Acad Sci 1987.

Please correlate with clinical conditions.

Method:- TWO-STEP IMMUNOENZYMATIC (SANDWICH) ASSAY
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